

Annex 2: MANAGEMENT OF SUSPECTED, PROBABLE AND CONFIRMED COVID-19 CASE

1. OVERALL MANAGEMENT OF COVID-19 IN MALAYSIA

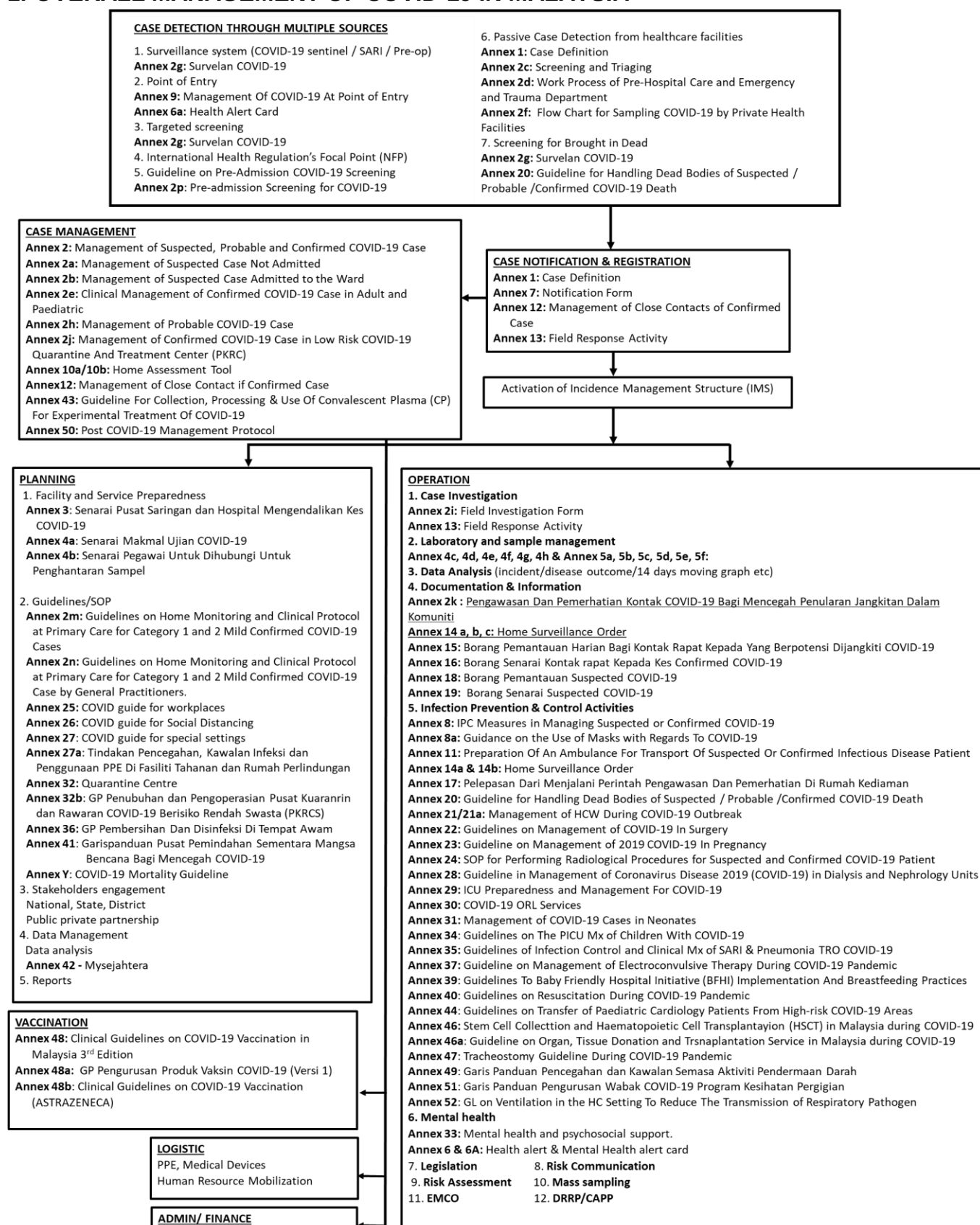


Figure 1: Overall management and response of COVID-19 cases in Malaysia.

Updated on 15 January 2022

2. MANAGEMENT OF SUSPECTED, PROBABLE AND CONFIRMED COVID-19 CASE

2.1 Criteria for Hospital Admission

- a. Confirmed COVID-19 patients (laboratory confirmed case) shall be admitted to the hospital if fulfil any of the following criteria.
 - i. All category 3 to 5 disease
 - ii. Those found to be unstable after evaluation in COVID-19 assessment centres (category 2 moderate – see Annex 2m).
 - iii. Uncontrolled comorbidity such as DKA, Hypertensive emergency, unstable angina etc.
 - iv. Category 1 to 2, End Stage Renal Failure (ESRF) on dialysis if alternate outpatient dialysis arrangements are not available.
 - v. Immunocompromised¹
 - vi. Age < 1 years old (category 3 and above) or paediatric with significant comorbidity² (Category 2 and above)
 - vii. Pregnant mothers in category 2 moderate and above.
 - viii. Pregnant mothers with BMI ≥ 35 kg/m² at booking.
 - ix. Pregnant mothers who are not fully vaccinated with medical /obstetrics morbidities, **regardless of COVID category**.
 - x. Unsuitable to be managed while on home quarantine.

Refer to Annex 2m (Guidelines on Home Monitoring and Clinical Protocol at Primary Care) and Annex 23a (Guidelines on Management of COVID-19 in Obstetrics) for suitability of home quarantine.

Confirmed COVID-19 cases can be admitted directly to low risk COVID-19 quarantine and treatment centers if deemed stable, after discussion with relevant physician.

- b. Suspected or Probable COVID-19 cases who are clinically ill³. Suspected or probable case who are not clinically ill but are not suitable for home surveillance can be admitted to quarantine station.

¹ Includes solid or bone marrow transplant recipients, people with cancer undergoing active chemotherapy, cancers of the blood and bone marrow, primary immunodeficiency, HIV infected with low cd4 count and not on suppressive ART therapy, splenectomised individuals, on prolonged corticosteroids or other immunosuppressive.

²Current evidence suggests that children with **medical complexity**, with **genetic, neurologic, or metabolic conditions**, or with **congenital heart disease** can be at increased risk for severe illness from COVID-19.

Children with **obesity, diabetes, asthma or chronic lung disease, thalassaemia, sickle cell disease, or immunosuppression** can also be at increased risk for severe illness from COVID-19 (CDC 2021; NIH 2021; RCPCH 2021). **Refer Appendix 1 for List of Comorbidities for Paediatrics.**

³The clinical condition of the patient is based on clinical judgement of the clinician in-charge.

3. CONFIRMED CASE OF COVID-19

All confirmed COVID-19 cases need to be:

- a. Isolated either at home, quarantine center, PKRC or hospital.
- b. Notified as COVID-19 case to District Health Office (PKD) as soon as possible.
- c. Managed clinically as per recommendation in Annex 2e: Clinical Management of Confirmed COVID-19 Case in Adult and Paediatrics.
- d. Field Investigation by District Health Office (PKD) as per Annex 13: Field Response Activity.

4. CRITERIA FOR TRANSFER TO A STEP-DOWN FACILITY

- i. **Confirmed case of COVID-19** can be transferred to a step-down facility if fulfil the following criteria:
 - a. Asymptomatic or improved symptoms.
 - b. Stable vitals and no hypoxia.
 - c. Stable comorbid if any.
 - d. Improved or stable laboratory data including inflammatory markers (especially C-reactive protein or lactate dehydrogenase).
 - e. Do not have ongoing clinical needs such as haemodialysis.
 - f. Not suitable for home isolation
- ii. Step-down can be to an identified ward in district hospital or an area which is suitable within the acute hospital.
- iii. The coordination and management of this facility is under the responsibility of the hospital.
- iv. Daily monitoring by medical personnel must be done in this facility.

5. CRITERIA FOR ENDING ISOLATION PRECAUTIONS

Clinical category	Vaccination status	Period of isolation ^{1, 2}	Testing requirements before discharge	Other requirements
Category 1 (asymptomatic)	Unvaccinated	10 days	Nil	
	2 or 3 doses completed / 1 dose completed for single-dose vaccine	7 days		
Category 2-3, not severely immunocompromised ³	Irrespective of vaccine status	10 days	Nil	At least 24 hours have passed since resolution of fever without the use of fever-reducing medications. And Other symptoms such as dyspnea, cough have improved.
Category 4-5, not severely immunocompromised	Irrespective of vaccine status	10 – 20 days	Nil	
Severely immunocompromised ³	Irrespective of vaccine status	Decided on case-to-case basis	RTK Ag test negative at least once	
1. Period of isolation is from the date of onset of illness (for cat 2-5) and from the date of positive COVID-19 test: date of sample taken (for cat 1) 2. Admitted patients can be discharged once clinically stable to complete their isolation at home 3. Severe immunocompromising conditions and treatments include but are not limited to: a. Active treatment for solid tumor and hematologic malignancies b. Receipt of solid-organ transplant and taking immunosuppressive therapy c. Receipt of CAR-T-cell therapy or hematopoietic cell transplant (HCT) (within 2 years of transplantation or taking immunosuppression therapy) d. Moderate or severe primary immunodeficiency (e.g., DiGeorge syndrome, Wiskott-Aldrich syndrome) e. Advanced or untreated HIV infection (people with HIV and CD4 cell counts <200/mm³, history of an AIDS-defining illness without immune reconstitution, or clinical manifestations of symptomatic HIV) f. Active treatment with high-dose corticosteroids (i.e., ≥20 mg prednisone or equivalent per day when administered for ≥2 weeks), alkylating agents, antimetabolites, transplant-related immunosuppressive drugs, cancer chemotherapeutic agents classified as severely immunosuppressive, tumor necrosis factor (TNF) blockers, and other biologic agents that are immunosuppressive or immunomodulatory.				

Note:

- a. COVID-19 cases fulfilling the criteria for ending isolation precautions above but still requiring ongoing inpatient care such as stroke rehabilitation can be discharged from COVID care and transferred to the appropriate ward.
- b. Category 5 patients in ICU, who still require ICU care beyond 20 days of illness, can also be discharged from COVID care. This is based on data that infectious viruses have not been isolated beyond day 20 of illness even in those critically ill.

6. CRITERIA FOR DISCHARGE TO CONTINUE HOME MONITORING

Patient can also be considered for home isolation until the end of their isolation period if ongoing care needs can be met at home and;

- Home condition is suitable for home isolation.
- Patient can be monitored by virtual CAC or other modalities until the end of the isolation period.
- Patient is able to come back to the hospital, if there is any deterioration in medical condition.

7. ARRANGING FOR PATIENT TRANSFER HOME TO COMPLETE THE ISOLATION PERIOD

- a. Patient should have a personal vehicle for transportation and cannot use public transport.
- b. High risk individuals such as unvaccinated, elderly, those with multiple comorbid, pregnant or immunocompromised should not be the designated driver.
- c. The patient should be given clear instructions on what to do when they leave the ward to minimise risk of exposure to staff, patients, and visitors on their way to their transport. The transport is not allowed to stop anywhere enroute home.
- d. Need to ensure patient has a digital or written home surveillance order with expected date of release from isolation.
- e. The patient and the driver should wear facemask for the duration of the journey.
- f. The patient should sit in the back of the vehicle with as much distance from the driver as possible.
- g. Vehicle windows should be (at least partially) open to improve ventilation.
- h. Vehicles should be cleaned at the end of the journey. Cleaning includes areas that patient have been in direct contact with and surfaces near to where the patient was sitting.
- i. Ensure the patient has a supply of tissues and a waste bag for disposal for the duration of the journey; the waste bag should then be taken into their house, put into another waste bag and held for a period of 72 hours before disposal with general household waste.

8. POST DISCHARGE PLAN FOR CONFIRMED COVID-19 CASE

- a. For patients with co-morbidities, to arrange appointment for the follow-up at the nearest health facilities and to ensure adequate supply of medications until the next appointment.
- b. For children with significant co-morbidities, please ensure they have enough medications upon discharge. Get a new appointment with the respective primary team for follow-up post completion of quarantine period.
- c. Upon discharge, all patients should be provided with a complete **COVID-19 Patient's Discharge Note (Appendix 2)** and **Health Education Pamphlet: Guideline for COVID-19 Patient Discharged from Hospital (Appendix 3)**.

List of comorbidities for paediatrics: *(If in doubt, please contact the paediatrician)*

- i. Immunodeficiency:
 - Child on immunosuppressive drugs;
 - HIV with CD4 <50 or had opportunistic infections over last 6 months;
 - Primary immunodeficiency (e.g., Severe combined immunodeficiency (SCID), Chronic granulomatous disease (CGD), Bruton's disease)
- ii. Child with swallowing problem, impaired cough or airway clearance (e.g., Cerebral palsy)
- iii. Children who are life-dependent on long term ventilation (home oxygen therapy, BIPAP, CPAP, tracheostomy)
- iv. Obese/syndromic child/genetic syndrome/delayed development with **medically complex diseases**
- v. Uncontrolled diabetes mellitus
- vi. Thalassemia with severe iron overload
- vii. Post kidney transplant (first 3 months or on immunosuppressant)
- viii. Congenital heart disease (single ventricle pathology, cyanotic heart disease not operated, symptomatic heart failure)
- ix. Severe persistent bronchial asthma
- x. Other chronic respiratory problems: repaired congenital thoracic abnormality,
 - a. Cystic fibrosis, bronchopulmonary dysplasia, bronchiectasis, interstitial lung
 - b. disease, bronchiolitis obliterans
- xi. Chronic gastroenterology problems: inflammatory bowel disease, child on home TPN, decompensated liver disease, active or relapsed autoimmune liver disease.





GARIS PANDUAN DISCAJ PESAKIT COVID-19 DARI HOSPITAL

Risalah ini bertujuan untuk menyampaikan informasi dan nasihat kesihatan kepada pesakit COVID-19 yang telah sembuh dan dibenarkan pulang ke rumah.

APAKAH YANG ANDA PERLU LAKUKAN SEMASA DI RUMAH ?



- Pastikan anda berehat secukupnya.



- Minum air untuk kekalkan hidrasi.



- Segelintir pesakit mungkin masih mengalami keletihan, lenguh badan, batuk dan sesak nafas ketika berjalan atau menaiki tangga yang mungkin berlaku setelah radang paru-paru yang serius.



- Pastikan pengudaraan bilik dalam keadaan baik dengan membuka tingkap.

ADAKAH SAYA BOLEH KEMBALI BEKERJA ?



Ya, anda boleh kembali bekerja sebaik sahaja cuti sakit anda berakhir.

PERKARA YANG ANDA PERLU TAHU

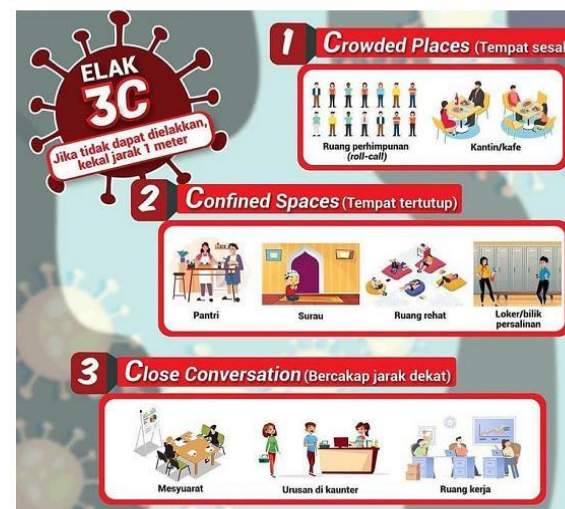
Apa yang kita tahu mengenai COVID-19

- Anda tidak lagi berada dalam tempoh bahaya penyakit.
- Kebarangkalian untuk anda menjangkiti orang lain adalah sangat rendah atau tiada setelah anda menamatkan tempoh pengasingan seperti yang dinasihatkan oleh doktor.

Perkara yang kita belum pasti

- Tempoh perlindungan antibodi setelah pulih dari jangkitan COVID-19 masih tidak diketahui. Jadi anda dinasihatkan untuk mengamalkan konsep 'Elak 3C, Amalkan 3W' (Sila rujuk gambarajah di sebelah).

ELAKKAN 3C, AMALKAN 3W



NASIHAT AM



Praktikkan penjarakan sosial sekurang-kurangnya 1 meter dari satu sama lain.



Pastikan memakai topeng muka jika perlu berada secara dekat.



Cuci tangan dengan kerap menggunakan sabun dan air atau 'hand sanitizer'.



Menutup mulut dan hidung sekiranya batuk atau bersin.



Amalkan kebersihan diri.



Pastikan pengambilan makanan yang berkhasiat dan air yang mencukupi.



Ahli keluarga yang berumur 65 tahun ke atas dinasihatkan untuk lebih berwaspada dan mengambil berat langkah-langkah pencegahan yang disarankan.



Dinasihatkan untuk tidak menerima atau menghadkan tetamu di rumah.



Jauhi diri daripada tempat yang sesak.

ADAKAH EMOSI SAYA STABIL ?

Gangguan emosi kerap kali berlaku semasa dan setelah jangkitan COVID-19.

Anda dinasihatkan untuk meminta bantuan sekiranya terdapat simptom-simptom berikut:

- Gangguan emosi
- Kerisauan melampau
- Kesukaran tidur atau konsentrasi
- Putus asa



BAGAIMANA UNTUK MEMINTA PERTOLONGAN SEKIRANYA MENGALAMI GANGGUAN EMOSI ?

Anda boleh merujuk ke fasiliti kesihatan berdekatan sama ada klinik atau hospital. Anda juga boleh menghubungi talian hotline dibawah untuk tindakan selanjutnya.

COVID-19



Talian Sokongan Psikososial
COVID-19



• 011-63996482
• 011-63994236
• 03-29359935

Waktu operasi setiap hari : 8 pagi sehingga 5 petang

Perkhidmatan ini dikendalikan oleh petugas psikologi KKM dan sukarelawan MERCY.

SEGERA DAPATKAN BANTUAN



Sila berjumpa doktor yang berdekatan atau hubungi hospital yang telah memberi rawatan COVID-19 sebelum ini sekiranya anda mengalami gejala seperti berikut:

1. Anda mengalami demam semula
2. Anda mengalami sesak nafas atau sesak nafas sedia ada menjadi semakin teruk
3. Anda mengalami sakit dada atau sakit dada sedia ada menjadi semakin teruk
4. Tidak boleh makan atau minum



Hospital : _____



No Telefon : _____

#KitaTeguhKitaMenang
#StrongerTogether



GUIDELINES FOR COVID-19 PATIENT DISCHARGED FROM HOSPITAL

This brochure is designed to provide information and advice for COVID-19 patients after going home.

WHAT SHOULD YOU DO AT HOME AFTER DISCHARGE ?



- Once you get home, rest adequately.
- Stay hydrated, and get plenty of sleep.
- You may continue to feel fatigue, have body aches, cough, and shortness of breath on walking or climbing stairs, which can occur after a serious viral pneumonia.
- Make sure the room is well ventilated by opening the windows.

CAN I GO BACK TO WORK ?



Yes, you can return to work immediately after your sick leave is over.

INFORMATION YOU SHOULD KNOW

What we know about COVID-19

- You are over the period of danger.
- The risk of you spreading to those close to you is considered minimal or nil once you have completed the isolation period as advised by our doctor.

What we don't know for sure yet

- The duration of protection after COVID-19 infection is uncertain yet. Therefore, you should always follow the good practice of 3C and 3W (refer to diagrams beside).

AVOID 3C, PRACTICE 3W



GENERAL ADVICE



Practice social distancing of at least 1 meter from others.



Required to wear face mask when in close contact with people.



Wash your hands with soap and water or use hand sanitizer regularly.



Cover your mouth and nose whenever you cough or sneeze.



Maintain good personal hygiene and cleanliness.



Keep good hydration and nutrition.



Consider extra protection for household members over 65 years or with underlying illness.



Please avoid or limit the number of visitors to your house.



Avoid going to crowded areas.

AM I COPING WELL ?

Emotional disturbance is common during and after COVID infection.

Please ask for help if you experience symptoms below:

- Mood disturbances
- Frequent worries
- Difficulty sleeping or concentrating
- Feel hopelessness and restlessness
- Suicidal thought



HOW DO I ASK FOR HELP ?

You can see your family doctor, or doctor at the nearest health clinic or hospital.
You also can call the hotlines below for further assistance.

COVID-19



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SEEK MEDICAL ATTENTION



See your nearest doctor immediately or contact the hospital you were discharged from if you develop any of the following symptoms:

1. New onset of fever
2. New onset or worsening shortness of breath
3. New onset or worsening of chest pain
4. Unable to tolerate orally



Hospital : _____



Contact No: _____

#KitaTeguhKitaMenang
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